ADULT ACUTE BURN RESUSCITATION FOR BURNS >20% TBSA	
Patient arrival to trauma bay	**Fluid resuscitation calculation**
Establish definitive airway if needed (TBSA >40% or symptomatic inhalation injury)	Use LR
Establish vascular access	Initial fluid rate (mL/hr) = 10 x _____% TBSA
Screen for other injuries	
Estimate % TBSA (2nd and 3rd degree burns only)	___________________
Estimate ideal body weight	
Place urinary catheter	
Prevent hypothermia	

Clearly document the following information	
Date and time of injury:	**TBSA:**
Date and time of admission to Burn ICU:	**Inhalation injury (Y/N):**
Weight (kg):	**Preadmission fluids (make sure it is documented in EPIC):**
Ideal body weight (kg):	

Colloid initiation
Trigger volume for colloid initiation:
(50 mL x ideal body weight) ____ mL
Includes any volume given pre-hospital and in ED.
When adding colloid:
A: Initial 5% albumin IV infusion rate: Total fluid rate at the time trigger volume is met x (1/3) = _____________ mL/hr
B: Adjust LR infusion rate as follows: Total fluid rate at the time trigger volume is met x (2/3) = _____________ mL/hr

UOP <30 ml/hr	**UOP 30-50 ml/hr**	**UOP >50 ml/hr**
Increase fluid rates by 20%	Maintain current rate.	Decrease fluid rates by 20%
(includes LR and albumin, if running)	If UOP remains 30-50 ml/hr for 2 consecutive	(includes LR and albumin,
Call attending for two consecutive	hours, decrease fluid rates by 10% (includes LR	if running)
hours of UOP <30 ml/hr.	and albumin, if running)	

Burn resuscitation reminders/tips

- Trend bladder pressure every 4 hours in burns >30% TBSA.
- Titrate both colloid and crystalloid hourly. Minimum total fluid rate = 195 ml/hr unless otherwise instructed.
- If coagulopathy present, escharotomy required, or there is refractory shock, discuss FFP use with attending.
- Do not bolus IV fluids unless there is impending cardiovascular collapse.
- Do not use POCUS to assess intravascular volume status during burn resuscitation. If patient with inappropriately normal HR, eval for cardiac depression due to burn shock.
- Do not use diuretics to increase UOP.
- For hypotension (MAP <65), assess for missed injury and begin vasopressors. Vasopressin at 0.04 units/min (non-titratable) as first line. Add Norepinephrine 0.05 mcg/kg/min if necessary (0.01-1 titratable). Replete calcium.
- Persistent oliguria: AKI may have occurred. Evaluate shock and volume status with other data (base deficit, LA, CVP, SVV). If intravascular volume appears adequate despite oliguria, STOP increasing fluids (discuss with attending).
- Strongly consider CRRT for refractory shock, acidemia, volume overload.

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Do Not File in Medical Record

Patient Name: Patient Identification #: